FAX TRANSMITTAL

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Contents
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1. Endocrinology -- Metastatic Lymphadenopathy & Thyroid Tissue Removal
2. Gastroenterology -- Laparoscopy, Laparotomy, & Cholecystectomy
3. General Medicine -- Sleep Study Followup

Specialty: Endocrinology
Sample Name: Metastatic Lymphadenopathy & Thyroid Tissue Removal

TITLE OF OPERATION: , Central neck reoperation with removal of residual metastatic lymphadenopathy and thyroid tissue in the central neck. Left reoperative neck dissection levels 1 and the infraclavicular fossa on the left side. Right levels 2 through 5 neck dissection and superior mediastinal dissection of lymph nodes and pretracheal dissection of lymph nodes in a previously operative field.,INDICATION FOR SURGERY: , The patient is a 37-year-old gentleman well known to me with a history of medullary thyroid cancer sporadic in nature having undergone surgery in 04/07 with final pathology revealing extrafocal, extrathyroidal extension, and extranodal extension in the soft tissues of his medullary thyroid cancer. The patient had been followed for a period of time and underwent rapid development of a left and right infraclavicular lymphadenopathy and central neck lymphadenopathy also with imaging studies to suggest superior mediastinal disease. Fine-needle aspiration of the left and right infraclavicular lymph nodes revealed persistent medullary thyroid cancer. Risks, benefits, and alternatives of the procedures discussed with in detail and the patient elected to proceed with surgery as discussed. The risks included, but not limited to anesthesia, bleeding, infection, injury to nerve, lip, tongue, shoulder, weakness, tongue numbness, droopy eyelid, tumor comes back, need for additional treatment, diaphragm weakness, pneumothorax, need for chest tube, others. The patient understood all these issues and did wish to proceed.,PROCEDURE DETAIL: ,After identifying the patient, the patient was placed supine on the operating room table. The patient was intubated with a number 7 nerve integrity monitor system endotracheal tube. The eyes were protected with Tegaderm. The patient was rotated to 180 degrees towards the operating surgeon. The Foley catheter was placed into the bladder with good return of urine. Attention then was turned to securing the nerve integrity monitor system endotracheal tube and this was confirmed to be working adequately. A previous apron incision was incorporated and advanced over onto the right side to the mastoid tip. The incision then was planned around the old scar to be excised. A 1% lidocaine with 1 to 100,000 epinephrine was injected. A shoulder roll was applied. The incision was made, the apron flap was raised to the level of the mandible and mastoid tip bilaterally all the way down to the clavicle and sternal notch inferiorly. Attention was then turned to performing the level 1 dissection on the left. Subsequently the marginal mandibular nerve was identified over the facial notch of the mandible. The facial artery and vein were individually ligated and marginal mandibular nerve traced superiorly and perifascial lymph nodes freed from the marginal mandibular nerve. Level 1A lymph nodes of the submental region were dissected off the mylohyoid and digastric. The submandibular gland was appreciated and retracted laterally. The mylohyoid muscle appreciated. The lingual nerve was appreciated and the submandibular ganglion was ligated. The hypoglossal nerve was appreciated and protected and digastric tunnel was

then made posteriorly and the lymph nodes posterior along the marginal mandibular nerve and into the parotid gland were then dissected and incorporated into the specimen for histopathologic analysis. The marginal mandibular nerve stimulated at the completion of this portion of the procedure. Attention was then turned to incising the fascia along the clavicle on the left side. Dissection then ensued along the floor of the neck palpating a very large bulky lymph node before the neck was identified. The brachial plexus and phrenic nerve were identified. The internal jugular vein identified and the mass was freed from the floor of the neck with careful dissection and suture ligation of vessels. Attention was then turned to the central neck. The strap muscles were appreciated in the midline. There was a large firm mass measuring approximately 3 cm that appeared to be superior to the strap musculature. A careful dissection with incorporation of a portion of the sternal hyoid muscle in this area for a margin was then performed. Attention was then turned to identify the carotid artery and the internal jugular vein on the left side. This was traced inferiorly, internal jugular vein to the brachiocephalic vein. Palpation deep to this area into the mediastinum and up against the trachea revealed a 1.5 cm lymph node mass. Subsequently this was carefully dissected preserving the brachiocephalic vein and also the integrity of the trachea and the carotid artery and these lymph nodes were removed in full and sent for histopathologic analysis. Attention was then turned to the right neck dissection. A posterior flap on the right was raised to the anterior border of the trapezius. The accessory nerve was identified in the posterior triangle and traced superiorly and inferiorly. Attention was then turned to identifying the submandibular gland. A digastric tunnel was performed back to the sternocleidomastoid muscle. The fascia overlying the sternocleidomastoid muscle on the right side was incised and the omohyoid muscle was appreciated. The omohyoid muscle was retracted inferiorly. Penrose drain was placed around the inferior aspect of the sternocleidomastoid muscle. Subsequently the internal jugular vein was identified. The external jugular vein ligated about 1 cm above the clavicle. Palpation in this area and the infraclavicular region on the right revealed a firm irregular lymph node complex. Dissection along the floor of the neck then was performed to allow for mobilization. The transverse cervical artery and vein were individually ligated to allow full mobilization of this mass. Tissue between the phrenic nerve and the internal jugular vein was clamped and suture ligated. The tissue was then brought posteriorly from the trapezius muscle to the internal jugular vein and traced superiorly. The cervical rootlets were transected after the contribution, so the phrenic nerve all the way superiorly to the skull base. The hypoglossal nerve was identified and protected as the lymph node packet was dissected over the internal jugular vein. The wound was copiously irrigated. Valsalva maneuver was given. No bleeding points identified. The wound was then prepared for

Specialty: Endocrinology
Sample Name: Metastatic Lymphadenopathy & Thyroid Tissue Removal

closure. Two number 10 JPs were placed through the left supraclavicular fossa in the previous drain sites and secured with 3-0 nylon. The wound was closed with interrupted 3-0 Vicryl for platysma, subsequently a 4-0 running Biosyn for the skin, and Indermil. The patient tolerated the procedure well, was extubated on the operating room table, and sent to the postanesthesia care unit in good condition.

Specialty: Gastroenterology
Sample Name: Laparoscopy, Laparotomy, & Cholecystectomy

PREOPERATIVE DIAGNOSES: , Cholelithiasis, cholecystitis, and recurrent biliary colic.,POSTOPERATIVE DIAGNOSES: , Severe cholecystitis, cholelithiasis, choledocholithiasis, and morbid obesity.,PROCEDURES PERFORMED: , Laparoscopy, laparotomy, cholecystectomy with operative cholangiogram, choledocholithotomy with operative choledochoscopy and T-tube drainage of the common bile duct.,ANESTHESIA: , General.,INDICATIONS: , This is a 63-year-old white male patient with multiple medical problems including hypertension, diabetes, end-stage renal disease, coronary artery disease, and the patient is on hemodialysis, who has had recurrent episodes of epigastric right upper quadrant pain. The patient was found to have cholelithiasis on last admission. He was being worked up for this including cardiac clearance. However, in the interim, he returned again with another episode of same pain. The patient had a HIDA scan done yesterday, which shows nonvisualization of the gallbladder consistent with cystic duct obstruction. Because of these, laparoscopic cholecystectomy was advised with cholangiogram. Possibility of open laparotomy and open procedure was also explained to the patient. The procedure, indications, risks, and alternatives were discussed with the patient in detail and informed consent was obtained.,DESCRIPTION OF PROCEDURE: , The patient was put in supine position on the operating table under satisfactory general anesthesia. The entire abdomen was prepped and draped. A small transverse incision was made about 2-1/2 inches above the umbilicus in the midline under local anesthesia. The patient has a rather long torso. Fascia was opened vertically and stay sutures were placed in the fascia. Peritoneal cavity was carefully entered. Hasson cannula was inserted into the peritoneal cavity and it was insufflated with CO2. Laparoscopic camera was inserted and examination at this time showed difficult visualization with a part of omentum and hepatic flexure of the colon stuck in the subhepatic area. The patient was placed in reverse Trendelenburg and rotated to the left. An 11-mm trocar was placed in the subxiphoid space and two 5-mm in the right subcostal region. Slowly, the dissection was carried out in the right subhepatic area. Initially, I was able to dissect some of the omentum and hepatic flexure off the undersurface of the liver. Then, some inflammatory changes were noted with some fatty necrosis type of changes and it was not quite clear whether this was part of the gallbladder or it was just pericholecystic infection/inflammation. The visualization was extremely difficult because of the patient's obesity and a lot of fat intra-abdominally, although his abdominal wall is not that thick. After evaluating this for a little while, we decided that there was no way that this could be done laparoscopically and proceeded with formal laparotomy. The trocars were removed.,A right subcostal incision was made and peritoneal cavity was entered. A Bookwalter retractor was put in place. The dissection was then carried out on the undersurface of the liver. Eventually, the gallbladder was identified, which was markedly scarred down

and shrunk and appeared to have palpable stone in it. Dissection was further carried down to what was felt to be the common bile duct, which appeared to be somewhat larger than normal about a centimeter in size. The duodenum was kocherized. The gallbladder was partly intrahepatic. Because of this, I decided not to dig it out of the liver bed causing further bleeding and problem. The inferior wall of the gallbladder was opened and two large stones, one was about 3 cm long and another one about 1.5 x 2 cm long, were taken out of the gallbladder.,It was difficult to tell where the cystic duct was. Eventually after probing near the neck of the gallbladder, I did find the cystic duct, which was relatively very short. Intraoperative cystic duct cholangiogram was done using C-arm fluoroscopy. This showed a rounded density at the lower end of the bile duct consistent with the stone. At this time, a decision was made to proceed with common duct exploration. The common duct was opened between stay sutures of 4-0 Vicryl and immediately essentially clear bile came out. After some pressing over the head of the pancreas through a kocherized maneuver, the stone did fall into the opening in the common bile duct. So, it was about a 1-cm size stone, which was removed. Following this, a 10-French red rubber catheter was passed into the common bile duct both proximally and distally and irrigated generously. No further stones were obtained. The catheter went easily into the duodenum through the ampulla of Vater. At this point, a choledochoscope was inserted and proximally, I did not see any evidence of any common duct stones or proximally into the biliary tree. However, a stone was found distally still floating around. This was removed with stone forceps. The bile ducts were irrigated again. No further stones were removed. A 16-French T-tube was then placed into the bile duct and the bile duct was repaired around the T-tube using 4-0 Vicryl interrupted sutures obtaining watertight closure. A completion T-tube cholangiogram was done at this time, which showed slight narrowing and possibly a filling defect proximally below the confluence of the right and left hepatic duct, although externally, I was unable to see anything or palpate anything in this area. Because of this, the T-tube was removed, and I passed the choledochoscope proximally again, and I was unable to see any evidence of any lesion or any stone in this area. I felt at this time this was most likely an impression from the outside, which was still left over a gallbladder where the stone was stuck and it was impressing on the bile duct. The bile duct lumen was widely open. T-tube was again replaced into the bile duct and closed again and a completion T-tube cholangiogram appeared to be more satisfactory at this time. The cystic duct opening through which I had done earlier a cystic duct cholangiogram, this was closed with a figure-of-eight suture of 2-0 Vicryl, and this was actually done earlier and completion cholangiogram did not show any leak from this area.,The remaining gallbladder bed, which was left in situ, was cauterized both for hemostasis and to burn off the mucosal lining. Subhepatic and

Specialty: Gastroenterology
Sample Name: Laparoscopy, Laparotomy, & Cholecystectomy

subdiaphragmatic spaces were irrigated with sterile saline solution. Hemostasis was good. A 10-mm Jackson-Pratt drain was left in the foramen of Winslow and brought out through the lateral 5-mm port site. The T-tube was brought out through the middle 5-mm port site, which was just above the incision. Abdominal incision was then closed in layers using 0 Vicryl running suture for the peritoneal layer and #1 Novafil running suture for the fascia. Subcutaneous tissue was closed with 3-0 Vicryl running sutures in two layers. Subfascial and subcutaneous tissues were injected with a total of 20 mL of 0.25% Marcaine with epinephrine for postoperative pain control. The umbilical incision was closed with 0 Vicryl figure-of-eight sutures for the fascia, 2-0 Vicryl for the subcutaneous tissues, and staples for the skin. Sterile dressing was applied, and the patient transferred to recovery room in stable condition.

Specialty: General Medicine
Sample Name: Sleep Study Followup

REASON FOR VISIT:, This 48-year-old woman returns in followup after a full-night sleep study performed to evaluate her for daytime fatigue and insomnia.,HISTORY OF PRESENT ILLNESS: , The patient presented initially to the Pulmonary Clinic with dyspnea on minimal exertion. At that time, she was evaluated and found to have evidence for sleep disruption and daytime fatigue. She also complained of nocturnal choking episodes that have since abated over the past several months. In the meantime, she had been scheduled for an overnight sleep study performed to evaluate her for sleep apnea, returns today to review her study results.,The patient's sleep patterns consist of going to bed between 9.00 and 10.00 p.m. and awakening in the morning between 5.00 and 6.00 a.m. She reports difficulty in initiating sleep and then recurrent awakenings every 1 to 2 hours throughout the night. She reports tossing and turning throughout the night and awakening with the sheets in disarray. She reports that her sleep was much better quality in the sleep laboratory as compared to home. When she awakens, she might have a dull headache and feels tired in the morning. Her daughter reports that she has heard the patient talking during sleep and snoring. There are no apneic episodes. The patient reports that she used to cough a lot in the middle of the night, but has no longer been doing so in recent weeks.,During the daytime, the patient reports spending a lot of sedentary time reading and watching TV. She routinely dozes off during these sedentary activities. She also might nap between 2.00 and 3.00 p.m., and nods off in the evening hours.,The patient smokes perhaps one to two packs of cigarettes per day, particularly after dinner.,She reports that her weight has fluctuated and peaked at 260 pounds approximately 1 year ago. Since that time, her weight is down by approximately 30 pounds.,The patient is managed in Outpatient Psychiatry and at her Maintenance Clinic. She takes methadone, trazodone, and Seroquel.,PAST MEDICAL HISTORY:,1. Depression.,2. Hepatitis C.,3. Hypertension.,4. Inhaled and intravenous drug abuse history.,The patient has a history of smoking two packs per day of cigarettes for approximately 25 pounds. She also has a history of recurrent atypical chest pain for which she has been evaluated.,FAMILY HISTORY: , As previously documented.,SOCIAL HISTORY: ,The patient has a history of inhalation on intravenous drug abuse. She is currently on methadone maintenance. She is being followed in Psychiatry for depression and substance abuse issues. She lives with a room-mate.,REVIEW OF SYSTEMS:, Not contributory.,MEDICATIONS: , Current medications include the following:,1. Methadone 110 mg by mouth every day.,2. Paxil 60 mg by mouth every day.,3. Trazodone 30 mg by mouth nightly.,4. Seroquel 20 mg by mouth nightly.,5. Avalide (irbesartan) and hydrochlorothiazide.,6. Albuterol and Flovent inhalers two puffs by mouth twice a day.,7. Atrovent as needed.,FINDINGS: , Vital Signs: Blood pressure 126/84, pulse 67, respiratory rate 18, weight 232 pounds, height 5 feet 8 inches, temperature 97.4 degrees, SaO2 is 99 percent on room air at rest.

Specialty: General Medicine
Sample Name: Sleep Study Followup

HEENT: Sclerae anicteric. Conjunctivae pink. Extraocular movements are intact. Pupils are equal, round, and reactive to light. The nasal passages show deviation in the nasal septum to the right. There is a slight bloody exudate at the right naris. Some nasal mucosal edema was noted with serous exudate bilaterally. The jaw is not foreshortened. The tongue is not large. Mallampati airway score was 3. The oropharynx was not shallow. There is no pharyngeal mucosa hypertrophy. No tonsillar tissue noted. The tongue is not large. Neck is supple. Thyroid without nodules or masses. Carotid upstrokes normal. No bruits. No jugular venous distention. Chest is clear to auscultation and percussion. No wheezing, rales, rhonchi or adventitious sounds. No prolongation of the expiratory phase. Cardiac: PMI not palpable. Regular rate and rhythm. S1 and S2 normal. No murmurs or gallops. Abdomen: Nontender. Bowel sounds normal. No liver or spleen palpable. Extremities: No clubbing or cyanosis. There is 1+ pretibial edema. Pulses are 2+ in upper and lower extremities. Neurologic: Grossly nonfocal.,LABORATORIES:, Pulmonary function studies reportedly show a mild restrictive ventilatory defect without obstruction. Diffusing capacity is well preserved.,An overnight sleep study was performed on this patient at the end of 02/07. At that time, she reported that her sleep was better in the laboratories compared to home. She slept for a total sleep time of 398 minutes out of 432 minutes in bed (sleep proficiency 92 percent). She fell asleep in the middle of latency of less than 1 minute. She woke up after sleep onset of 34 minutes. She had stage I sleep that was some elevated at 28 percent of total sleep time, and stage I sleep is predominantly evident in the lateral portion of the night. The remainders were stage II at 69 percent, stage III and IV at 3 percent of total sleep time.,The patient had no REM sleep.,The patient had no periodic limb movements during sleep.,The patient had no significant sleep-disordered breathing during non-REM sleep with less than one episode per hour. Oxyhemoglobin saturation remained in the low to mid 90s throughout the night.,Intermittent inspiratory flow limitation compatible with snoring was observed during non-REM sleep.,ASSESSMENT AND PLAN: , This patient presents with history of sleep disruption and daytime sleepiness with fatigue. Her symptoms are multifactorial.,Regarding the etiology of difficulty in initiating and maintaining sleep, the patient has a component of psychophysiologic insomnia, based on reports of better sleep in the laboratory as compared to home. In addition, nontrivial smoking in the home setting may be contributing significantly to sleep disruption.,Regarding her daytime sleepiness, the patient is taking a number of long-acting central nervous system acting medications to sedate her and can produce a lasting sedation throughout the daytime. These include trazodone, Seroquel, and methadone. Of these medications, the methadone is clearly indicative, given the history of substance abuse. It would be desirable to reduce or discontinue trazodone

Specialty: General Medicine
Sample Name: Sleep Study Followup

and then perhaps consider doing the same with Seroquel. I brought this possibility up with the patient, and I asked her to discuss this further with her psychiatrist.,Finally, to help mitigate sleep disruption at night, I have provided her with tips for sleep hygiene. These include bedtime rituals, stimulus control therapy, and sleep restriction as well as avoidance of nicotine in the evening hours.

LABORATORY REPORT

Packet: 005

Analyte	Value	Units	Flag
Glucose	142	mg/dL	High
Hemoglobin	11.8	g/dL	Low
WBC	12.4	10*3/uL	High
Sodium	136	mmol/L	
Potassium	4.2	mmol/L	
Creatinine	1.4	mg/dL	High
BUN	24	mg/dL	High
Platelets	228	10*3/uL	
Hemoglobin A1c	7.6	%	High
TSH	2.1	mIU/L	